

LRI ED Injury Streaming **Adult Walk-In VAC Tool**

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Suitable presentations

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- Limb injury, including burn
- Limb soft tissue conditions
- Head injury
- Suspected blunt c-spine injury
- Eye conditions
- Non-traumatic low back pain

Specific exclusions for head injury

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- Still not fully alert and orientated
- >1 episode of vomiting since the injury
- Current treatment with anticoagulants:
 - Warfarin
 - Rivaroxaban
 - Apixaban
 - Edoxaban
 - Dabigatran
 - Therapeutic LMWH
- Any sign of basal skull fracture
- Post-traumatic seizure
- Clear evidence of open or depressed skull fracture
- New focal neurological deficit

General exclusion criteria

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- Potential airway compromise
- Uncontrolled external bleeding
- Dangerous injury mechanism
- Significantly abnormal vital signs
- Systemic infectious illness suspected
- Suspected hip or femoral shaft fracture
- Frailty or significant comorbidities requiring more than injury care
- Systemic condition causing injury
 - e.g. hypoglycaemia, seizure or TLOC
- Non-mobile and bariatric, including need for hoisting
- Displaying violent/aggressive behaviour
- Altered behaviour due to the influence of drugs and/or alcohol
- Acute mental health issues/agitation
- Cognitive impairment and risk of wandering/falls unless with a carer

Plain film imaging

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If you have a VAC doctor or ANP, or if time permits, please request X-rays (not CT!) from VAC and sent patient to imaging on the way to Injuries. Patients really resent having to walk back from Injuries for their X-rays...

Triage tasks may include

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eObs, analgesia, dressings, X-rays, bloods, burn first aid etc.

